Crashing Medical Patient

AM-07 | Adult Medical

Quick flow

01 Crashing Patient? [EMT / AEMT / Paramedic]

Hypoxia/ventilatory failure • severe distress • SBP <90/shock • declining mental status • peri-arrest dysrhythmia

02 Assign Roles + Bring Equipment [EMT / AEMT / Paramedic]

Leader • airway • monitor/defibrillator • IV/IO/medications • hemorrhage • documentation

03 Immediate Threats [EMT / AEMT / Paramedic]

Airway/ventilation • hemorrhage • tension pneumothorax • hypoglycemia • anaphylaxis • dysrhythmia • shock

04 Access + Cause-Directed Care [AEMT / Paramedic]

Rapid IV/IO • reassessed fluids/vasopressors • pads early • use the linked definitive pathway

05 Move + Reassess [EMT / AEMT / Paramedic]

Move when lifesaving care is underway • reassess at least every 5 min during prolonged extrication

Overview

- Use for immediate or impending physiologic collapse requiring simultaneous resuscitation, rapid decisions, and expedited transport.
- Begin immediately lifesaving care at the patient side when safe, then move promptly once critical interventions are underway.

Provider-Level Actions

Higher levels include preceding-level actions.

EMT

- Assign team roles; obtain a full vital set within 5 minutes when feasible without delaying lifesaving care.
- Position, suction, use BVM ventilation and oxygen when indicated, control hemorrhage, apply AED/monitor support, prevent heat loss, request resources early, and prepare transport.

AEMT

- Establish IV access or proceed rapidly to IO when urgent access is needed and IV is unsuccessful or unlikely.
- Give cause-appropriate isotonic fluid with reassessment and use a supraglottic airway when indicated.

Paramedic

- Provide advanced airway/ventilation with waveform capnography, dysrhythmia treatment, vasopressors, and cause-specific resuscitation.
- Select the closest appropriate destination and consider Covenant Health Air only when it improves time or capability without delaying movement.

Indications

- Immediate or impending physiologic collapse: SpO_2 below 90% or inadequate ventilation, severe respiratory distress/exhaustion, SBP below 90 or shock, rapidly declining mental status, poor perfusion, extreme respiratory rate, or peri-arrest dysrhythmia.

Contraindications

- Do not remain on scene when safety is threatened, definitive treatment is unavailable, hemorrhage requires surgery or blood products unavailable to the crew, or transport/rendezvous is the fastest route to needed care.
- Do not use NIPPV for vomiting, facial trauma, inability to cooperate, inability to protect the airway, or impending arrest.

Assessment | Immediate assessment

- Assess airway, ventilation, oxygenation, pulse, BP/perfusion, severe hemorrhage, glucose, temperature when indicated, and reversible causes.
- A normal GCS or initially acceptable BP does not exclude a crashing patient; reassess after every intervention.

Treatment steps

- Bring all anticipated resuscitation equipment to the patient and assign team functions.
- Treat airway, ventilation, severe hemorrhage, tension pneumothorax, hypoglycemia, anaphylaxis, dysrhythmia, and shock before prolonged movement when safe.
- Prioritize BVM ventilation and oxygenation. Use advanced airway management when the patient cannot protect the airway or cannot be adequately oxygenated/ventilated less invasively; confirm with waveform capnography.
- Identify peri-intubation hypotension and use AM-05 fluid/vasopressor treatment before high-risk intubation when possible.
- Use AM-05 for fluid and norepinephrine strategy; avoid indiscriminate crystalloid in uncontrolled hemorrhage, pulmonary edema, cardiogenic shock, or volume overload.
- Apply pads early when instability is present or anticipated and use the linked cardiac pathway for dysrhythmia.
- Move promptly once critical interventions are underway; plan reassessment stops during prolonged extrication and reassess at least every 5 minutes.

Medications

Push-Dose Epinephrine

Temporary bridge only - open the dedicated reference for the approved indication, labeled 10 mcg/mL preparation, weight-based pediatric dose, and two-bolus limit.

- Use the medication reference rather than preparing or dosing from memory.
- Begin a vasoactive infusion when indicated and contact Medical Control before any dose beyond the two-bolus limit.

Warnings / precautions

- GCS 15 does not equal stability.
- Do not allow procedures to delay definitive care.
- Fluids and vasopressors support perfusion but do not replace correction of the cause.
- Use NIPPV only when the patient can protect the airway and is not vomiting or peri-arrest.

Clinical pearls

- The goal is not prolonged scene care; it is to avoid moving an unresuscitated patient through a difficult extrication when immediate EMS interventions can improve stability.
- Call for additional hands and advanced resources early.

Related pathways / contacts

Push-Dose Epinephrine - Medication Reference - Open the dedicated reference for the approved preparation, dosing, and two-bolus limit.

UP-01 Universal Patient Care

AM-05 Hypotension / Shock

AR-01 Adult Airway Management

AC-03 Adult Cardiac Arrest

AM-01 Allergic Reaction / Anaphylaxis

TB-06 Multiple Trauma

TE-07 Overdose / Toxic Ingestion

Source and references

Tennessee EMS ALS/BLS Blended Protocol Guidelines | North Carolina College of Emergency Physicians AM-07 source protocol | Claiborne County EMS formulary and medical-director decisions, October 2026.

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